



DEPARTMENT OF RADIODIAGNOSIS & IMAGING

Patient Name:	SAMARTH SEETHARAM HOSMANE				Sex:	M	Age:	10Y 7M 1D
Patient ID:	3980016	Order ID:	RT262932	Location:				
REFERRING PHYSICIAN:		DR VASUDEVA BHAT K						
REFERRING DEPARTMENT:		PDO001						
STUDY PART		EXAMINATION DESCRIPTION				Performed on		
		MR Brain				01-08-2025/09:45:08		
CLINICAL DETAILS:		Glioblastoma grade 4, post resection and multiple cycles of CTRT, on follow up						

RADIOLOGY REPORT:

FOLLOW UP PLAIN AND CONTRAST MRI STUDY OF BRAIN **(Previous plain and contrast MRI study of brain done on 13.02.2025)**

IMPRESSION:

Known case of thalamic glioblastoma (Grade 4), post near total excision on 11.01.2025, on adjuvant chemotherapy (Temozolomide) and radiotherapy (last cycle completed in April 2025), current scan shows:

Burr hole defect is noted involving the frontal bone on the left side.

Few foci of blooming in the periphery of the resection cavity within right thalamus (likely hemosiderin deposition-**likely sequelae of previous operative procedure.**

Resection cavity involving the right thalamus along the atrium and body of the right lateral ventricle extending posteriorly along the tectum, third ventricle and cerebral aqueduct - **persists the same in size with smooth peripheral enhancement and facilitated diffusion and elevated choline/creatinine ratio and reduced NAA on MRS.**

Nodular enhancement along the postero-inferior and medial aspects of the resection cavity (i.e.; along the tectum and third ventricle, aqueduct) with low rCBF and **ASL and elevated choline/creatinine ratio and reduced NAA on MRS- suspicious for residual tumor-persists.**

Ill-defined altered signal intensity area appearing T1 hypointense and T2/FLAIR hyperintense measuring ~3.2 x 2.5 x 2.8 cm (CC x TR x AP) antero-supero-lateral to the resection cavity showing patchy areas of diffusion restriction with heterogenous enhancement- **likely pseudo-progression-persists nearly the same in size with significant reduction of adjacent vasogenic edema.**

No significant midline shift.

A focal well-defined T1 isointense, T2/FLAIR hyperintense lesion not showing diffusion restriction/blooming and showing no significant post contrast enhancement measuring ~2.0 x 2.6 x 1.5 cm (AP x TR x CC) showing elevated choline/creatinine ratio and reduced NAA on MRS is noted involving the cerebellar vermis and medial aspect of the right cerebellar hemisphere causing compression of the fourth ventricle with mild retrograde prominence of the third and bilateral lateral ventricle and periventricular seepage-**likely metastases.(New finding)**

No other significant interval changes as compared to the previous study.